Historical research report

Round 2: Portfolio reset and proof shortlist

Review date: 4 November 2026 | Original report: 26 pages

Historical snapshot

This file preserves an earlier review. Later findings and the current selection supersede its recommendations, rankings and proposed next actions. Read the current shortlist and Round 4 before using this report for a development decision.

Current assessment

Current shortlist
github.com/KyleJKistner/ddiscov/blob/main/decisions/current-shortlist.md

Round 4 research and decisions
github.com/KyleJKistner/ddiscov/blob/main/research/rounds/round4/README.md

Original report preserved

All original report pages follow unchanged. The original report page numbers are retained; this archival cover is additional.

PORTFOLIO RESEARCH | DECISION REPORT

ddiscov
A broader search.
A concrete proof shortlist.

4 November 2026 UTC | KyleJKistner/ddiscov | Snapshot c241dc4c6cfa

First funding priority: human muscle force rescue through selective PIK3C2B inhibition, using BLU3797 as the defined starting compound. The independent challenge supports a staged experiment. It combines causal genetic evidence, an exact small molecule, severe-disease functional rescue and a newly named human assay route. The important unanswered question is human force at a usable tissue exposure. [B1-B6,B8]

Parallel priority: AAT during donor-lung perfusion. Its direct human-organ evidence is stronger, but new work must demonstrate durable repair after washout and transplantation or another material product improvement. Repeating the existing perfusion result would add too little. [A1-A8]

Proof priority	Development question
1. BLU3797 / PIK3C2B	Restore established human muscle force with neural and hepatic exposure separation.
2. AAT / donor lungs	Sustain clinically meaningful organ repair after free drug is removed.
3. CPTX / incomplete SCI	Replicate durable coordinated function after delayed injury without harmful excitation.
4. mIGF / cornea	Beat topical insulin, or deliver equivalent useful repair with a compelling dosing advantage.
5. FeM1269 / iron restriction	Deliver sequestered human macrophage iron into erythroid hemoglobin during persistent restriction.
6. PLC-zeta / activation failure	Produce a stable defined reagent that adds normal development beyond ionophore.

What changed: all 622 records across six collections now have a traceable review disposition; three domain teams generated 30 proposals, consolidated to 29 distinct proposals, and root added a separate POLG/sulbactam lead. Three independent challenge reviews tested the strongest recommendations. This supersedes the earlier contact-first priority order.

S-tier assessment: BLU3797 is the closest current candidate for an exceptional proof investment. No asset yet meets every S-tier development criterion on public evidence. The report defines exactly what would earn promotion; the label is not manufactured to satisfy a search target.

What was reviewed, and what the counts mean

Collection	Records	Interpretation
Historical topics	232	Topics include molecules, families and process notes.
Component proposals	70	68 concepts; related components and combinations remain distinct.
Functional comparisons	67	57 concepts; repeated comparisons are not independent discoveries.
Broad screen	34	Additional platforms and delivery/assembly ideas.
Later research screen	167	166 distinct source IDs; one repeated ID remains traceable.
Formulation comparisons	52	50 marketed-product rows plus two boundary cases; 48 underlying families.
Total traceable records	622	Full row-level coverage, not 622 unique drugs or deep reviews.

The complete JSON register preserves the original row, source file, identifiers, links and every domain review. There are 743 review assignments because useful overlaps are retained. The 825 historical occurrences and 35 atlas families provide context; they are not added to the drug count. [R1-R5]

Review depth is explicit

Every row received repository-level triage. Deeper work was concentrated on functional candidates, current results that change rank, exact material identities and the leading development questions. The three domain ledgers cover 308, 183 and 224 records, respectively; root closed 28 initially unmapped entries, some subsequently overlapping another team. These counts must not be summed as independent candidates.

The blood/ex vivo review contains 10 deep primary reviews and 13 primary follow-ups; the barrier review distinguishes 143 repository screens from selected deeper checks; the neuro/metabolic review marks eight freshly rechecked existing records plus its generated leads. The attached register retains these original depth labels rather than flattening everything into a misleading “vetted” flag.

Limits of the public repository

The public decisions page leaves investment order open and says the latest three came from successive search rounds. It does not contain a definitive A-tier ranking. The first audit already reviewed the 28-file public snapshot and numerical-method limitations. The original private archive behind 116 fingerprints was not supplied; this report does not claim to replay its complete research history. [R2,R6,R7]

No emails, owner discussions, purchases, wet-lab studies or remote GitHub changes were performed. The included patch is a concrete process update prepared against the pinned snapshot. The scientific selection does not depend on any requested reply.

02 / DIAGNOSIS

The process weakness is selection, not just sourcing

1. Recency and uneven scrutiny can manufacture a winner

A recent lead arrives with a favorable abstract and few recorded weaknesses. An older one accumulates caveats. Comparing those summaries promotes ignorance. The reset applies a common challenge to finalists and recovers older candidates such as CPTX, FeM1269 and PLC-zeta. The previous three are now ordinary entries in the register. [R2,R4]

2. “Available” had too many meanings

Separate public identity, research material, qualified model, exposure/product fit and eventual commercial rights. A disclosed peptide can be synthesizable without a clinical product. A catalog protein can be the wrong isoform. A live sponsor program can make a new generic experiment redundant without making all useful research impossible. No single yes/no field represents these states.

3. A functional mechanism can still fail the intended product

Count useful force, transport, filtration, coordinated movement or durable barrier repair. Keep markers, immediate challenge prevention and microscopic local mechanics in their proper roles. BPG10 local cartilage stiffness is not whole-joint restoration; removing histamine is not demonstrated control of every anaphylaxis mediator; animal corneal closure is not human chronic denervation repair. [D4,D5,M1]

4. Evidence must not migrate between constructs or studies

The public record contains several traps: the AAT abstract and article likely describe one donor set; CPTX-expressing cells are a separate product from injected protein; hinokitiol patient-cell data do not validate FeM1269; the MBP-PLC-zeta construct does not authenticate a later clinical preparation. These are identity and independence errors, not small editorial details. [A1,A2,C3,F5,P1,P2]

5. The comparator that could invalidate the product must come first

For mIGF, include insulin. For AAT, optimize and protein-match the perfusate. For platelet additives, compare with modern prolonged cold storage. For enzyme combinations, measure benefit over each component and the current treatment, not just vehicle. A product that loses to a simple available alternative should lose priority early. [M5,D2]

6. Search should end each pass with a decision

A new paper should produce an advance condition, a stop condition, a narrower indication, a comparator update or a reason to retain the record at screening depth. “Interesting; contact author” is not a completed scientific assessment. Equally, calling everything uncertain without choosing a funded experiment is not a useful portfolio decision.

The replacement operating process

Stage	Required output
Define the lesion and product	Exact entity, indication/genotype, compartment, treatment window and useful function.
Normalize the evidence	Independent biological units; direct comparator; exposure; effect and uncertainty; dataset overlap; adverse evidence.
Establish a research route	Public structure/sequence or exact manufactured product; a plausible sourcing route; a named functional model with qualification needs.
Find the decisive gap	One result that changes whether development should continue. It must add information beyond the best existing study.
Independent challenge	A separate reviewer seeks the strongest confounder, active comparator, current negative result and construct mismatch.
Choose and limit the proof	Fund qualification first; release later work only after interpretable function and exposure are demonstrated.
Update the portfolio	Advance, redesign, retain as comparator, reserve or stop. Every disposition carries its reason and review depth.

A practical decision rule

Prefer an experiment when a positive result would materially increase the case for a useful product, a negative result would stop or redirect it, and the cost and execution burden are bounded. Consider those outcomes together. A low-cost experiment that cannot change a decision is poor value; a more demanding human-organ study can be worthwhile if it answers a product-defining question.

This is a qualitative expected-value framework. Numerical probabilities are not assigned because the source record does not calibrate them. Evidence from independent genetics, a drug and human physiology should move confidence more than three publications repeating one dataset.

Commercial facts remain specific and proportionate

Record public patent families, sponsor activity and known competitors. Do not infer freedom to operate from an abandoned application, or commercial rights from a supplier listing. Do not turn uncertain private terms into the prerequisite for public scientific selection. Any later transaction has its own claim-specific work; none is initiated here.

What earns S-tier

An exceptional candidate needs meaningful established-disease rescue; independent or orthogonal support; a reproducible material and feasible exposure; a credible advantage over current care; no unresolved fatal safety contradiction at the required exposure; and a compelling next milestone with plausible value capture. BLU3797 has enough of this package to justify the first proof, but its human functional and tissue-window questions remain open.

BLU3797: the leading new proof investment

Why this is more than a single attractive mouse paper

Independent kinase-dead genetics restored muscle force in Mtm1-deficient mice; BIN1 disease did not respond, defining an important boundary. A separate 2016 inducible-deletion experiment begun after symptoms supported reversibility. The 2026 exact compound improves severe-model survival and motor performance, including a late-start experiment. These interventions converge on a specific MTM1 hypothesis. [B1,B2,B8]

The drug study still has small groups, juvenile treatment timing, selected cell-clone biology and no demonstrated mature human force rescue. Its plasma pharmacology does not establish the free muscle/brain window. Those gaps are precisely why a human function experiment can add substantial value. The result should not be sold as evidence across all centronuclear myopathies. [B1,B2]

Both the compound and a first functional model are identifiable

Item	Publicly established starting point
Compound	Published stereodefined structure and synthesis; Probechem PC26519 listing. Packs are quotation-based; current physical stock and lot quality were not confirmed. [B1,B4]
Human tissue	Curi Bio CB-SKM-XLMTM: male patient-derived MTM1 p.L463S line, with vendor-reported force and fatigue deficits. [B5,B6]
Critical limitation	The catalog explicitly calls it a standalone patient line. An unrelated healthy control is not an isogenic correction. A single donor does not establish broad human rescue. [B6]
Research contribution	A mature human target-engagement/force curve, allelic boundaries and tissue exposure constraints could be useful for BLU3797 or a properly distinguished successor.

The central risk is a tissue window

Human PIK3C2B loss-of-function variants and experimental findings implicate the target in neuronal excitability and epilepsy susceptibility. In the muscle genetic study, partial inactivation did not normalize force. Together, these observations make muscle engagement with CNS sparing a central design requirement. They do not prove that reversible BLU3797 treatment will cause seizures. [B2,B3]

Hepatobiliary function should be an early parallel measurement. The Unite-CNM DYN101 experience shows that tolerability difficulties in this population extend beyond AAV platforms; that is not evidence that this compound is hepatotoxic. [B9]

Decision

Fund model qualification and the human force/exposure experiment first. It can resolve a large unanswered development question with a named compound and a named starting model. Do not spend the first major tranche on another mouse wire-hang replication.

BLU3797: the experiment and promotion rule

First release: establish an interpretable human result

Authenticate the compound against its published identity. Qualify a stable force/fatigue deficit in differentiated tissue before assigning treatment. Include MTM1 restoration or correction in the same background to establish that the measured deficit is disease-related and rescuable; use unrelated healthy tissue as a reference, clearly labeled. Establish repeatability across independent differentiations and casting batches.

Randomize mature tissues after recording baseline function. Compare drug, vehicle, an orthogonal PIK3C2B perturbation and MTM1 restoration. A less potent related inhibitor is not an inactive control. Measure total force, force normalized to viable contractile tissue, fatigue/recovery, relaxation, calcium handling and injury. This separates stronger muscle from simply more muscle or a differentiation effect.

Measure free exposure and target engagement alongside function. Link neuronal-network and hepatobiliary functional testing to the same exposure scale. Normal viability alone is too weak a safety readout. One positive donor is sufficient to earn expansion, not to establish population-wide efficacy.

Second release: confirm breadth and a usable window

Replicate in additional independent backgrounds, including complete-loss and residual-function MTM1 variants where feasible. Add a focused unbound muscle/brain distribution study to connect in vitro effects to a plausible administered exposure. Prespecify a meaningful functional rescue margin from the qualified model and its genetically restored range before blinded efficacy analysis. Do not pick a threshold after seeing the drug result.

Result	Decision
Human force and fatigue recover; exposure separates from neural/hepatic dysfunction	Advance the exact compound into focused translational and product work. This is the strongest route to S-tier promotion.
Orthogonal target inhibition works, but BLU3797 distribution or compound-specific toxicity is poor	Advance the target into an explicitly separate CNS-sparing chemistry program. A new molecule must earn its own evidence.
The model lacks a stable MTM1-rescuable deficit	Stop the assay and fix model qualification; do not mislabel this as drug failure.
Qualified human models repeatedly fail despite target engagement, or necessary inhibition is inseparable from relevant harm	Stop this development direction rather than extend the screening narrative.

Why the result is transferable

The output would be an authenticated reference article, a qualified human assay, a force/exposure relationship, genotype boundaries, orthogonal target validation and a safety constraint. Those are useful development facts even before a product transaction. Their value depends on data quality and actual differentiation, not on the number of positive images.

Public prior art includes a Stemline application claiming a different named compound; its displayed abandoned status is not BLU3797 clearance. An exact structure/inventor claim search remains a defined later work item. It does not substitute for the proposed experiment. [B7]

AAT: the strongest human-organ alternative

The human report used eight declined donor lung pairs, with left/right lungs randomized to AAT or control circuits. Oxygenation, compliance and edema-related measures favored treatment. The 2020 abstract and 2026 article appear to describe the same eight-donor experiment and are counted once pending contrary provenance. Full effect-size tables and the eight-donor dose were not available through the retrieved article route. [A1,A2]

A separate pig EVLP paper used Zemaira in perfusate, while a transplantation experiment administered AAT to recipients. The latter supports a different treatment route; it does not establish that ex vivo-only treatment produces lasting graft benefit after washout. [A3,A4]

Why a product is concrete

Zemaira is an FDA-approved plasma-derived A1PI product. It supplies a named manufactured starting component, although its approved indication is AAT deficiency rather than donor-lung repair. Its label does not validate prolonged warm recirculation or mixing into the intended perfusate. Functional activity, compatibility, aggregates, adsorption and processing stability are product work. [A5,A6]

The proposed product is a qualified repair additive/regimen for a specified perfusion workflow and injury phenotype. A useful result would define active exposure, identify responsive lungs and demonstrate retained function after the additive is removed. Simply adding a commercially manufactured protein and measuring another inflammatory marker is insufficient.

The most informative control

AAT changes the composition of the circuit. Extra protein, oncotic pressure, electrolytes or handling could account for some physiological benefit. Match those variables and compare with an optimized current perfusate. A mechanistic protease-activity measurement should accompany function; antigen concentration alone cannot establish an active enzyme inhibitor.

Existing work sets a higher bar

CSL has a public organ-perfusion patent application. Its human pilot describes five donor pairs and a specific higher concentration; that pilot must not be silently treated as the complete eight-donor clinical paper. Existing claims make the commercial landscape concrete, while leaving the scientific question of dose qualification and durability open. [A7]

A randomized regadenoson EVLP trial did not demonstrate better lung use or graft dysfunction outcomes. This supports demanding a clinically relevant durability bridge rather than assuming circuit physiology will become transplant benefit. It does not refute AAT. [A8]

Decision

Retain as the parallel proof priority. Its human functional base is stronger than BLU3797, but a new independent study must add durable repair, an improved dose specification or another clear product contribution. That requirement places it second for the next new research investment.

AAT: demonstrate repair that survives the circuit

Qualification before scarce donor organs

Establish functional AAT potency and stability throughout the intended warm recirculation period. Quantify loss to tubing, filters and tissue; distinguish inactive antigen from active inhibitor. Confirm formulation compatibility, oncotic matching and a mass balance through washout. This is a finite bench milestone that can fail before scarce human material is consumed.

Donor-paired physiology, then a transplant bridge

Use a blinded donor-paired comparison with an optimized protein/excipient-matched control. Account for asymmetric left/right injury using baseline measurements and prespecified allocation. Set a primary functional endpoint and a clinical acceptance rubric before the experiment. Oxygenation, compliance, fluid balance and barrier injury should tell a coherent story.

The donor is the independent unit. Hourly readings, biopsies and alveolar fields are repeated measurements. A low-dose strategy should retain useful function against a justified reference exposure; lower concentration alone does not create value. A factorial AAT/solnatide combination belongs only after the single-agent contribution is established.

The decisive step is a defined drug-free or recipient-reperfusion challenge followed by an ex vivo-only transplant validation. Track function across the period relevant to early graft injury. A recipient systemic-AAT arm can help explain mechanism, but cannot stand in for the intended ex vivo product.

Advance	Stop or redesign
Consistent donor-level improvement over optimized matched perfusion	Difference vanishes after correcting protein, electrolytes or baseline injury.
Benefit survives removal of free circuit drug	Benefit exists only while the circuit is continuously exposed.
Improved meaningful graft performance after transplant	Better edema/oxygenation on the machine fails to improve the graft.
A reproducible active concentration and compatible formulation	Potency declines unpredictably, or required product burden eliminates the practical advantage.

What the funded package would establish

A controlled component specification, a response criterion, an active-exposure range, durable function and an interpretable failure mode. That package would be useful to a transplant center, perfusion developer or AAT manufacturer. Their interest is an inference from the product use case, not an asserted partnership or a prerequisite email.

No quoted budget or secured organ access is claimed. The main execution burden is qualified paired-organ work and the transplant bridge, not finding the protein sequence. An assay or sequencing-service subtotal would substantially understate this project.

08 / RECOVERED LEAD

CPTX: recover useful circuitry after established injury

The 2020 protein study provides genuine mouse functional evidence, including delayed hemisection and acute contusion. Functional scoring was blinded. Protein became immunoblot-undetectable by day seven, while function was followed for six weeks. Those observations are attractive but do not establish that induced connections are reversible or harmless. [C1]

The patent adds delayed-contusion experiments at two and four weeks, with very small groups. This corrects an overly pessimistic claim that delayed contusion was never tested; it remains patent-only supporting evidence. The sequence and production configuration are public, and rehabilitation is already contemplated. A later CPTX-expressing cell study is a different product and did not show a significant incremental BBB score over control cells. [C2,C3]

The useful product question

Can a brief, spatially limited protein intervention restore coordinated activity in spared circuitry after incomplete injury without persistent pain, spasticity or autonomic disturbance? The intended indication should require surviving recruitable substrate. Rebuilding an anatomically disconnected chronic cord is a different proposition.

Proposed first proof

Make two independently qualified batches against the disclosed research configuration. Verify identity, oligomeric state, both binding activities and functional potency. Then run a delayed incomplete-contusion comparison on a fixed rehabilitation background, with allocation concealment, baseline severity strata and blinded analysis. Keep the initial design small enough to answer one question; do not introduce an unvalidated vector, hydrogel or cell graft at the same time.

Make coordinated movement primary, with conduction and an independent locomotor task as support. Retain all randomized animals in the primary analysis; walking-only subsamples are conditional outcomes. Quantify local distribution and exposure, and follow function and pathological excitation beyond detectable protein. Include an appropriate active or mechanism control without spreading power over an oversized combination matrix.

Advance condition	Stop condition
Replicated useful coordination and conduction after delayed injury	Only staining, reflexes or selected walking animals improve.
A usable spatial/exposure window and durable benefit	Persistent harmful excitation, pain or autonomic effects offset benefit.
Reproducible material using the public configuration	A different route/construct is needed and has not established equivalence.

Position: third major proof priority. The value is independent efficacy, safety and a product specification. Delayed dosing or rehabilitation alone is not a new invention. Known local delivery is enough for the first replication; clinical route simplification is a subsequent comparable-product task. [C2]

mIGF: make the retained peptide beat a real comparator

mIGF-Gel is a defined short biotin-capped peptide containing D-phenylalanine followed by YIGSSSR. The rat acute-injury result reports 48-hour closure of 94.1%, versus 86.1% with IGF1 and 76.9% with PBS, n=5. In dexamethasone-suppressed healing, once-daily gel was comparable to three-times-daily IGF1. This is useful retention/repair evidence, not proof in human chronic neurotrophic keratopathy. Detailed delayed-model denominators and blinding were not fully re-extracted in this pass. [M1,M2]

The product advantage must survive washout and insulin

Public chemistry supports independent synthesis, so an owner reply is not needed to define a research article. Nankai patent families identify existing claims. A generic “IGF peptide” is not an equivalent sample. Two lots should demonstrate identity, assembly, transparency and retained bioactivity before efficacy testing. [M3,M4]

Topical insulin has human treatment evidence and should be an active comparator. The strongest practical proposition may be durable repair with less dosing and better handling, rather than a small closure advantage over soluble IGF1. That proposition must be demonstrated rather than inferred from retention. [M5]

Proposed first proof

Use paired donor corneal epithelial tissue under repeated tear-like washout with a persistent repair limitation. Compare the exact gel, a physical gel control, validated insulin vehicle/formulation, free IGF1 and a clinical reference when the model can represent its function. Match delivered bioactivity and dosing burden explicitly. A scrambled sequence that no longer forms a gel is an incomplete physical control.

Assess closure together with barrier integrity, epithelial adhesion, viability and optical clarity. A subsequent defined denervation model is needed for a nerve-related claim; epithelial markers alone cannot establish restoration of innervation.

Advance	Stop
Better durable repair than insulin, or comparable repair with a substantial practical dosing/handling advantage	Only vehicle is beaten, or the benefit is just a larger delivered exposure.
Stable transparent material and strong repaired epithelium after washout	Haze, fragility, inconsistent assembly or practical instability consumes the advantage.

Position: fourth proof priority and a useful smaller local-product alternative. It has a crisp material and experiment, but its current comparative effect is modest and its most relevant disease translation is unproven. It should compete for resources on that basis rather than inherit a top label from attractive formulation images.

FeM1269: prove where the iron goes

FeM1269 is the optically positive (+)-6 tropolone derivative designed to reduce iron-complex aggregation. Mouse studies show anemia recovery, and the paper includes animal tolerability and exposure work. The efficacy model combines inflammation, dietary iron restriction and blood loss, with hepcidin declining by the endpoint. It supports post-induction recovery, but not yet rescue under persistent chronic inflammatory restriction. [F1,F2]

A public supplier listing and batch certificate make research sourcing concrete. The retrieved certificate does not establish the intended enantiomeric purity, which should be authenticated. The primary paper discloses Cajal-related patent interests. Hinokitiol patient-macrophage work supports the mechanism class but cannot be counted as exact FeM1269 human validation. [F3-F5]

The valuable human experiment

Track the same iron atoms from a sequestering human macrophage onto transferrin and into erythroid heme. Maintain the inflammatory/hepcidin restriction during the test. Compare authenticated FeM1269, hinokitiol, vehicle and a suitable iron-delivery reference, with controls that distinguish transport from chelation and nonspecific leakage.

Measure erythroid hemoglobinization and maturation alongside macrophage iron retention, transferrin-bound iron, labile/non-transferrin-bound iron, oxidative injury and viability. Use independent macrophage and erythroid donor backgrounds with an analysis that respects their crossed or nested structure. Numerous wells or donor pairings do not become independent patients.

Quantify actual medium and intracellular exposure. A buffer transport effect does not establish function in protein-rich tissue, and total mouse serum concentration cannot be treated as human free exposure. A follow-on animal study should maintain iron restriction while treating established anemia rather than repeat the optimized mixed model.

Advance	Stop
Useful heme incorporation and erythroid maturation across independent donors	Iron leaves macrophages but does not reach functional hemoglobin.
Benefit at a plausible measured exposure without harmful metal redistribution	Labile iron, aggregation or oxidative injury overlaps the effective range.
A sustained effect under persistent hepcidin restriction	Benefit depends on spontaneous resolution of inflammation or an unrealistically favorable assay.

Position: fifth proof priority. The chemistry and physical mechanism are unusually coherent. The next experiment can directly answer whether movement of iron becomes useful blood-cell function, making this a stronger research project than another general anemia biomarker study.

PLC-zeta: a real human signal with a reagent problem

A 2026 case series reports assisted activation in 135 couples allocated by suspected gamete deficit. Only 20 couples received rhPLC-zeta alone: two-pronuclear fertilization rose from 60/327 historical oocytes to 116/280 intervention oocytes. Five ongoing-or-delivered outcomes were reported among 17 transfer cycles. This is not five verified live births, and comparison with different ionophore-treated patients is not randomized superiority. [P1]

An arithmetic check changes how the headline is used

Printed counts	Review calculation
Dual treatment: 19 outcomes / 31 transfers	61.3%, rather than the printed 63.3%.
Ionophore: 21 outcomes / 39 transfers	53.8%, rather than the printed 54.8%.
rhPLC alone: 5 outcomes / 17 transfers / 32 intervention cycles	29.4% per transfer; 15.6% per intervention cycle. These are different denominators, not live-birth rates.

Calculations use the source tables. They do not erase the fertilization signal, but reduce confidence in headline pregnancy precision. Couples are independent units; cycles and eggs are clustered. Historical comparison also leaves regression to the mean and laboratory/cycle differences unresolved. [P1]

Define a reproducible product independently

The clinical paper names a custom 64.5-kDa GenScript preparation but does not provide a complete sequence/tag/lot specification. The separate 2024 MBP fusion and a shorter catalog PLCZ1 isoform are different materials. Matching a target name and molecular mass does not authenticate equivalence. Public sequence and production evidence nevertheless support building an explicit research reagent. [P1-P4]

The proposed product is a sequence-defined, stable recombinant reagent with a functional potency assay and a justified activation-deficiency selection rule. Its competitive value could be normal development in an ionophore-resistant subgroup, a wider controlled dose window or materially better handling. Calling it more physiological is not enough.

Proposed proof and stop rule

Qualify independent production lots for identity, aggregation, contaminants and activity. Test calcium oscillations, normal fertilization and subsequent development against ionophore in a relevant model with sibling-oocyte allocation and donor-level analysis. Then bridge the actual proposed product in appropriately consented human research material. A clinical live-birth claim would need its own prospective clinical evaluation.

Advance for reproducible developmental advantage or a defined comparator-resistant subgroup with a usable window. **Stop** if activation improves without development, potency is unstable or excessive calcium signaling causes harm. No current study establishes an S-tier differentiated product; the exact reagent comparison is still worth doing.

Three additional leads worth retaining

Plasminogen plus alteplase in kidney perfusion

The human study identifies a missing substrate in an acellular perfusate and shows removal of microvascular obstructions. Its overall program used 39 kidneys, but the central paired efficacy comparison was three donor pairs; urine-output improvement was not statistically conclusive. Clinical manufacturing precedents exist for both proteins. An earlier tPA-only trial is a different, less favorable comparator context. [K1-K3]

Proposed decision: in donor-blocked kidneys, require improved perfusion and filtration after washout, not merely cleaner histology or more urine. Measure residual fibrinolytic activity and injury. This is a strong reserve organ-repair experiment, below AAT because useful organ function is less securely established.

Sulbactam for selected POLG aggregation defects

An October 2026 paper identifies C4 as sulbactam and C1 as diltiazem. It reports mitochondrial and mouse motor rescue. The cell work primarily uses engineered lines, and the original knock-in model is relatively mild; the drug dose was not reconciled from retrieved methods. This is an obtainable-drug lead, but its exact exposure and human-function bridge remain open. [S1,S2]

Proposed decision: require endogenous patient/isogenic cellular rescue with meaningful muscle or hepatic function, measured feasible exposure and mitochondrial-genome integrity. Keep sulbactam separate from combination claims and from the different PZL-A activation mechanism. A failed human bridge should stop the repurposing thesis. [S3]

PslG selected by biofilm phenotype, paired with antibiotic

The 2026 study shows heterogeneous response among 14 clinical isolates, with nine disrupted by PslG. Biomass loss is not synonymous with killing. Earlier pulmonary work used favorable early timing and found increased bacteremia with an enzyme combination alone. These findings favor a phenotype-selected adjunct, not a universal biofilm dissolver. [G1,G2]

Proposed decision: measure viable organisms in both residual tissue and dispersed effluent after established infection. Require substantial added clearance over antibiotic alone at realistic local exposure, without surviving dissemination. A readily sampled local tissue indication may provide a cleaner initial proof than immediately committing to chronic inhaled delivery.

Why these stay below the core shortlist

Each has a concrete component or substrate and a useful next test. Kidney lysis has a small functional dataset; sulbactam has a model/exposure gap; PslG has a dissemination and endpoint problem. They remain active research options rather than being rejected for uncertain owner responsiveness.

Downgrades and retained alternatives

Family	Decision consequence
Fexlamose / MUC031 / AER01	Sponsor-reported 119-person Phase 2a failure on 30 October 2026 lowers confidence that ex vivo mucolysis predicts a useful COPD product. The full peer-reviewed dataset is not yet in this review. Do not generalize failure to every mucus mechanism. [D1]
Taurine for cold platelets	The CHIPS trial analyzed 989 recipients and found prolonged cold storage noninferior for cardiac-surgery hemostasis, with more reexploration. An additive must improve on that contemporary comparator, not just preserve a clotting assay for 21 days. [D2]
ABO-converted organs	A human decedent kidney avoided early hyperacute rejection, but antigen reappeared with antibody-mediated lesions beginning around day three. Durable compatibility is the missing product property. [D3]
BPG10 / cartilage charge replacement	Human local micromechanics are promising. Whole-cartilage fluid support, friction and loaded durability remain the decisive proof. PSS and PSS-PNIPAM data must stay attached to their respective constructs. [D4]
Engineered DAO	Histamine challenge, arterial enzyme administration versus IM adrenaline, early rescue timing and pooled mortality do not establish superiority in allergen-driven human anaphylaxis. Retain a mixed-mediator adjunct proof. [D5,D6]
Fosravuconazole / open-source mycetoma	The randomized fosravuconazole trial did not beat itraconazole. Open-source fenarimol chemistry is a practical research route, but larval survival does not yet establish mammalian grain penetration and cure. This is a science gap rather than a missing-email problem. [D7,D8]

Existing patient-model and engineering lanes remain visible

DAPT in MVID and SSP2518 in GUCY2C disease retain relevant patient physiology, with exposure and chronic safety constraints. Divertin remains a chemistry task. Local desmosomal rescue needs post-injury cohesion at delivered topical exposure. Haptoglobin has serious functional biology and a visible development landscape. All 52 formulation records are preserved, but an engineering question is not promoted until an exact product advantage is specified. Their detailed dispositions and source links remain in the register.

HJ02, icerguastat and ODAD1 were not selected as the first new funding priorities. Their previous scientific limitations remain documented, but they do not define the search space or its stopping rule. [R2]

New and materially revised proposals 1-6

Thirty distinct proposals remain after one duplicated BPG10/PRG4 concept was merged. The register classifies 19 as new to this repository, seven as refinements and four as untested combinations. These categories do not establish patent novelty or product readiness. Shared CoQ and AAT families are explicitly tagged.

Concept / type	Decisive next proof and disposition
Matrix-phenotype-selected PslG plus antibiotic BL-N01 new to repo	In established clinical-isolate biofilms, compare PslG, antibiotic, combination and inactive enzyme; quantify viable residual and dispersed organisms after physiological protease exposure and washout. **Decision:** Advance only if matrix selection predicts materially greater killing without increased viable dissemination. [N01]
Native DNase1L2 for actin-rich airway mucus BL-N02 new to repo	Across independent patient sputum donors, compare native DNase1L2 with dornase and an actin-resistant benchmark using mucus transport and rheology, not DNA digestion alone. **Decision:** Artificial mucus efficacy is insufficient for an inhaled product thesis. [N02]
Qualified topical insulin for neurotrophic epithelial defects BL-N03 new to repo	Compare a qualified formulation under tear-like washout in persistent epithelial repair models, measuring durable closure, barrier restoration and ocular handling stability. **Decision:** Highly obtainable and scientifically credible, but differentiation and existing formulation claims constrain discovery priority. [M5]
Retained topical losartan for established corneal fibrosis BL-N04 new to repo	Test established corneal scars against an appropriate active comparator at measured, tolerable stromal exposure; require durable transparency/mechanical improvement after washout. **Decision:** Retention must produce an incremental functional effect rather than repeat a weak broad efficacy claim. [N04]
Topical PDE4 inhibition for post-injury epithelial adhesion rescue BL-N05 refinement	After pathogenic-IgG injury in human skin or oral mucosa, measure tissue exposure, mechanical cohesion and barrier recovery with topical roflumilast versus relevant active therapy. **Decision:** The intended route must attain useful exposure and reverse established cohesion loss. [N05]
Defined dual KLK5/KLK7 cyclic peptide for local Netherton treatment BL-N06 new to repo	Select one sequence and compare barrier function, proteolysis and local exposure in SPINK5-deficient human epidermis against LEKTI/KLK benchmarks. **Decision:** Competition and incomplete article selection prevent near-term promotion. [N06]

These entries are proposed development designs. The complete JSON preserves exact components, demonstrated versus missing evidence, source URLs, original IDs and family overlap. Combination benefit is not assumed. A source supports the underlying observation, not proof that the proposed extension will succeed.

New and materially revised proposals 7-12

Concept / type	Decisive next proof and disposition
GM-CSF variant escaping pathogenic aPAP autoantibodies BL-N07 new to repo	First generate a sequence-defined variant and test receptor-dependent macrophage function across independent patient antibody preparations, including affinity-diverse controls. **Decision:** The crucial component must still be discovered; an epitope map is not a therapeutic construct. [N07]
Genotype-defined topical readthrough for SERPINB7 Nagashima keratosis BL-N08 new to repo	Use variant-defined patient epidermis to compare gentamicin with controls, linking full-length functional SERPINB7 restoration to barrier recovery rather than antibacterial effects. **Decision:** Modest human evidence warrants a bounded functional proof, not an S-tier claim. [N08]
BPG10 plus full-length recombinant PRG4 for cartilage mechanics BL-N09_BE-N07 untested combination	Paired human OA explants with both singles and combination under cyclic loading/washout, requiring incremental bulk fluid-load support and lower friction without impaired viability. **Decision:** Promote only if separate bulk and friction defects are corrected and combination outperforms the better single component. [N09]
Low-frequency mIGF-Gel tested against insulin under washout BL-N10 refinement	In paired corneal models with persistent repair limitation and repeated tear-like washout, compare exact mIGF, an appropriate physical gel control, insulin and free IGF1 on durable closure/barrier and optical quality. **Decision:** A meaningful reduction in dosing burden or superior durable repair against insulin would define a product; PBS superiority would not. [M2]
Controlled AAT additive for donor-lung repair during EVLP BE-N01 new to repo	Use donor-paired lungs and protein/excipient-matched optimized perfusion to test lower-dose active AAT and persistence after washout, followed by ex-vivo-only survival transplantation. **Decision:** Strong human physiological foundation and concrete supply; a new project must add durability, dose control or reproducibility, with known rights weighted separately. [A1]
Plasminogen replenishment plus alteplase in kidney NMP BE-N02 new to repo	Randomized contralateral kidneys comparing combination, component controls and optimized NMP, using filtered-tracer clearance and perfusion uniformity after washout. **Decision:** Clear substrate lesion and attainable components, but functional recovery must exceed histological plug clearance. [N12]

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New and materially revised proposals 13-18

Concept / type	Decisive next proof and disposition
Brief MAM exposure at renal reperfusion BE-N03 new to repo	Treat human donor kidneys at reperfusion with MAM versus DMM and vehicle under clinically realistic perfusion; require filtration/energy recovery after washout. **Decision:** Mechanistic rationale is strong enough to test, but human treatment function and context sensitivity remain decisive. [N13]
Local DiNAC lysis of VWF/platelet-rich obstructive clot BE-N04 new to repo	At measured physiological-plasma local exposure, test phenotype-defined patient clots under flow for restored passage, viable distal embolic burden and endothelial injury versus active lysis controls. **Decision:** Advance only with a defined susceptible clot phenotype and a useful local exposure/safety window. [N14]
Apotransferrin timed to post-transfusion iron release BE-N05 new to repo	In transfusion-relevant human material, compare timed apotransferrin with matched controls on NTBI neutralization plus a prespecified tissue or cellular functional injury endpoint. **Decision:** A short-window benefit may survive the negative chronic-anemia result, but needs direct functional demonstration. [N15]
Reversible calcium chelation plus taurine for cold platelet storage BE-N06 untested combination	Donor-blocked factorial storage study comparing each component and combination against current cold-storage practice, followed by recovery and hemostatic-function testing after calcium restoration. **Decision:** A storage biomarker or repeated twenty-one-day clotting result is insufficient differentiation. [N16]
DNase plus plasminogen to unlock retained tPA in resistant thrombi BE-N08 refinement	In resistant patient thrombi with measured retained tPA, compare DNase, plasminogen combination and additional fibrinolytic under flow, using reperfusion and embolic fragments rather than clot weight alone. **Decision:** A useful mechanistic refinement, but ex-vivo lysis cannot substitute for clinical recanalization. [N17]
Selective A2B blockade during EVLP BE-N09 new to repo	Independently repeat EVLP/transplant physiology with the exact benchmark, requiring a donor/graft functional gain beyond compliance alone and measuring exposure. **Decision:** Incomplete functional advantage and product specification keep this behind AAT. [N18]

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New and materially revised proposals 19-24

Concept / type	Decisive next proof and disposition
AAT plus solnatide for donor-lung barrier and fluid clearance BE-N10 untested combination	Only after confirming AAT-alone performance, use a donor-paired factorial EVLP comparison on sustained gas exchange, compliance and fluid clearance after washout. **Decision:** Do not add product complexity without superiority to the better single component. [A1]
BLU3797 for MTM1-deficient muscle NM-N01 new to repo	Mature human muscle force at measured exposure plus neural safety separation **Decision:** Potentially high-value functional rescue; prioritize human specific force and exposure/selectivity rather than another marker assay. [B1]
Transient CPTX protein plus timed rehabilitation after incomplete SCI NM-N02 refinement	Delayed contusion coordinated motor/conduction benefit after clearance without pathological excitation **Decision:** The valuable new contribution is delayed, transient protein benefit under an active rehabilitation comparison. [C1]
Post-onset cognate tRNA add-back in YARS1/GARS1 neuropathy NM-N03 refinement	Adult/post-onset function; neonatal prevention does not establish reversal **Decision:** Strong biology; differentiate through established-disease function and exact product rather than rediscovering the platform. [N22]
Exposure-matched piclidenoson in Lowe OCRL deficiency NM-N04 new to repo	Receptor-mediated uptake at human-plausible unbound kidney exposure; nominal organoid concentration is ~88x human total Cmax comparator **Decision:** Do not promote unless useful receptor-mediated rescue survives human-plausible exposure constraints. [N23]
Vanillic acid plus defined CoQ10 formulation in COQ9/selected COQ7 deficiency NM-N05 new to repo	Factorial human function, metabolic flux and feasible exposure; genotype restriction **Decision:** Chemically concrete and inexpensive to falsify; require incremental tissue function over optimized CoQ10. [N24]

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New and materially revised proposals 25-30

Concept / type	Decisive next proof and disposition
2,4-dihydroxybenzoate add-on in COQ8B nephropathy NM-N06 untested combination	Incremental human podocyte barrier function over optimized CoQ10 **Decision:** Do not count this as an independent platform win or generalize across COQ genotypes. [N25]
Defined phosphatidylcholine/ phosphatidylethanolamine for CMT1A NM-N07 new to repo	Mature human conduction plus lipid incorporation; no plasma-only surrogate **Decision:** Generic lecithin is not a new asset; measured neural incorporation and function could define a useful product. [N26]
GCN2iB benchmark for GARS1 neuropathy NM-N08 new to repo	Distinguish on-target benefit from ZAK inhibition and harmful low-dose GCN2 activation **Decision:** Retain as a mechanistic benchmark; a successor must separate GCN2 from ZAK and avoid an adverse low-exposure range. [N27]
Immobilized haptoglobin cartridge for CSF hemoglobin removal NM-N09 refinement	Paired patient-CSF clearance, vascular response, capacity, leachables and rebound; 8–12 independent samples proposed **Decision:** A tractable bench/ex-vivo step exists, but safe intracranial circulation is a later product question. [N28]
Sildenafil in genotype-defined ATP6 Leigh syndrome NM-N10 refinement	Human neural function at plausible free exposure then controlled motor/crisis outcomes **Decision:** Test exposure-matched human function before a controlled prospective disease study; do not claim a new molecule. [N29]
Sulbactam rescue of aggregation-prone POLG R-N01 new to repo	In patient-derived or isogenic human cells with endogenous, aggregation-prone POLG variants, demonstrate mtDNA maintenance and respiratory function at measured clinically plausible unbound sulbactam exposure, separating aggregation rescue from nonspecific stress effects. **Decision:** Do not outrank direct human organ or mature-muscle proofs until genotype-appropriate patient function and exposure are demonstrated. [S1]

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What I would fund, and what I would require back

Release the first proof in stages

First: BLU3797 reference-material and human-model qualification. Successful qualification earns post-differentiation efficacy and parallel neural/hepatic exposure work, followed by independent genotype replication. The spend buys a force/exposure answer, not another list of potential targets.

Parallel: AAT perfusate compatibility and functional-potency qualification. Scarce donor-organ work should follow only when optimized controls and a credible washout/durability design are ready. This keeps a direct human-physiology opportunity active without simply reproducing the published headline.

Next alternatives: CPTX is the strongest independent repair proof after those two. mIGF offers a local-formulation comparison; FeM and PLC-zeta offer sharply defined human-function or reagent questions. They stay in the shortlist with explicit conditions rather than being abandoned when one material detail is unresolved.

Require a transferable result package

Deliverable	Why it matters
Exact reference article and analytical release record	Another team can reproduce the material, not merely recognize the code name.
Qualified model with biological-unit accounting	A positive or negative result is interpretable and does not depend on pseudoreplication.
Blinded functional effect with active comparator	The product adds something useful beyond current practice or a cheaper available alternative.
Exposure and safety relationship	A biological effect can be connected to an achievable product configuration.
Full raw-data, analysis and deviation record	Independent challenge can test the result rather than rely on a presentation.
Advance, redesign or stop decision	Further funding depends on the prespecified question, not enthusiasm after one favorable endpoint.

Cost and execution discipline

No formal quotes were obtained, so this report does not attach invented precision to project budgets or delivery dates. Each proposed experiment has identifiable cost drivers: model generation/qualification, biological replicates, reference material, exposure analytics and specialist functional measurements. Those costs should be scoped as a complete milestone. An attractive price for a reagent or an isolated core service is not a total program budget.

The present decision is positive and bounded

There is enough evidence to put first proof money behind PIK3C2B inhibition in human MTM1 muscle. There is not yet enough evidence to label a particular finished drug an obvious S-tier asset. A strong force-and-window result would materially change that assessment; a clean negative result would prevent a much larger mistake. This is the decision the reset was designed to produce.

Reading the report and applying the update

ddiscov_portfolio_register.json is the complete working deliverable. It contains the 622-record source inventory with 743 review assignments, six final proof candidates, the 30 consolidated generated proposals, the decision policy and source metadata. Original screening depth and differing reviewer opinions are retained; final shortlist fields identify the integrated decision.

ddiscov_process_update.patch is prepared against snapshot c241dc4c6cfa. It adds a selection policy, a compact six-project snapshot and a link from the decisions page. It does not delete historical evidence or overwrite the source screens. No remote repository change or discussion was opened.

How to interpret uncertainty

“Not confirmed” describes this review, not impossibility. A supplier route is not received inventory; a disclosed sequence is not a qualified lot; a plausible human platform is not demonstrated drug efficacy. Conversely, a failed search for a current sponsor or a patent does not establish abandonment or freedom to operate.

Patent records are used to identify public claims and existing work, not to offer a jurisdiction-specific legal conclusion. Regulatory labels establish the named product and approved use, not approval of a proposed new workflow. Sponsor top-line releases are identified as such. Supplier model data are primary commercial claims requiring independent qualification.

Research limitations that matter

Some full texts and supplements were inaccessible to this retrieval route. AAT human dose/effect-size tables, portions of mIGF delayed-model methods, the full FeM chemistry/toxicology supplement and the exact 2026 DAO lot are examples. Those limits are recorded rather than filled with details from a different study. None was silently converted into a favorable finding.

The public record was checked as of 4 November 2026. This is a broad portfolio triage with targeted deep reviews and independent challenges, not exhaustive patent clearance or experimental replication of every historical paper. The first-pass whole-repository inspection and this complete row-level disposition together cover what is available in the public snapshot.

Source navigation

The following sources support the main report and first starting source for each generated concept. The JSON also preserves the fuller multi-source lists and original repository links. Reference IDs in square brackets resolve to the clickable entries below. Proposed study designs and prioritization are this review’s judgments, not claims that those experiments have already been performed.

Source register

[R1] Repository purpose and transferable-result milestone
Open source

[R2] Existing development comparisons; portfolio order left open
Open source

[R3] Historical and concept discovery collections
Open source

[R4] Later research screen index
Open source

[R5] Marketed-product formulation comparisons
Open source

[R6] Methods and limitations
Open source

[R7] Archive provenance boundaries
Open source

[B1] BLU3797 pharmacology and Mtm1 mouse rescue, JCI Insight 2026
Open source

[B2] Independent PIK3C2B kinase-dead genetic rescue, JCI Insight 2023
Open source

[B3] PIK3C2B loss of function and focal epilepsy, Brain 2022
Open source

[B4] BLU3797 PC26519 research product listing
Open source

[B5] Curi Bio XLMTM functional tissue model
Open source

[B6] Curi Bio cell-model catalog with MTM1 genotype
Open source

[B7] PIK3C2B inhibition patent landscape example
Open source

[A1] AAT in eight paired human donor lungs, 2026
Open source

[A2] Earlier eight-donor AAT conference abstract, 2020
Open source

[A3] AAT pig EVLP study, 2017
Open source

[A4] AAT recipient treatment in pig lung transplantation, 2016
Open source

Primary papers, regulatory records, institutional disclosures, public patents and supplier pages are identified by their titles. A source link does not imply that every claim on that page is endorsed.

Source register (continued)

[A5] FDA Zemaira product record
Open source

[A6] Zemaira prescribing information
Open source

[A7] CSL AAT organ-perfusion application and pilot details
Open source

[A8] Regadenoson randomized EVLP trial
Open source

[A9] Human solnatide lung physiology comparison
Open source

[C1] CPTX synaptic connector functional study, Science 2020
Open source

[C2] CPTX sequence and additional delayed-contusion patent examples
Open source

[C3] CPTX-expressing neural progenitors, separate cell product, 2024
Open source

[M1] mIGF-Gel corneal repair primary study, 2026
Open source

[M2] mIGF-Gel primary full-text record
Open source

[M3] Nankai peptide-gel patent application
Open source

[M4] Earlier Nankai peptide-gel patent
Open source

[M5] Multicenter topical-insulin cohort, 2026
Open source

[F1] FeM1269 primary paper, figures and disclosures, 2024
Open source

[F2] FeM1269 author manuscript and detailed model methods
Open source

[F3] FeM1269 research supplier listing
Open source

[F4] Public FeM supplier batch certificate
Open source

[F5] Hinokitiol patient-macrophage mechanism support, distinct compound
Open source

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Source register (continued)

[P1] PLC-zeta human assisted-activation case series, 2026
Open source

[P2] Separate MBP-PLC-zeta construct study, 2024
Open source

[P3] Canonical PLCZ1 sequence Q86YW0
Open source

[P4] Shorter PLCZ1 isoform/His-SUMO research product
Open source

[P5] Historical PLC-zeta application
Open source

[K1] Plasminogen plus tPA human kidney perfusion study
Open source

[K2] FDA Ryplazim plasminogen product record
Open source

[K3] Earlier tPA-only kidney perfusion randomized trial
Open source

[S1] Sulbactam/diltiazem POLG aggregation rescue, October 2026
Open source

[S2] Original Polg knock-in model characterization, 2021
Open source

[S3] PZL-A direct POLG activation comparator, 2025
Open source

[G1] PslG/PelA clinical-isolate comparison, 2026
Open source

[G2] Pulmonary biofilm hydrolase study and dissemination boundary
Open source

[D1] Fexlamose/MUC031/AER01 sponsor Phase 2a results, 30 October 2026
Open source

[D2] CHIPS cold-platelet randomized trial, 2026
Open source

[D3] ABO-converted kidney human decedent study
Open source

[D4] Human cartilage BPG10 mechanics study
Open source

[D5] Histamine-scavenging DAO study, 2026
Open source

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Source register (continued)

[D6] DAO heparin-binding-motif engineering, 2021
Open source

[D7] Fosravuconazole randomized mycetoma trial
Open source

[D8] Open-source mycetoma fenarimol chemistry
Open source

[B8] Post-symptomatic inducible Pik3c2b deletion, JCI 2016
Open source

[B9] Unite-CNM liver-function and DYN101 tolerability report
Open source

[N01] Starting evidence: Matrix-phenotype-selected PslG plus antibiotic
Open source

[N02] Starting evidence: Native DNase1L2 for actin-rich airway mucus
Open source

[N04] Starting evidence: Retained topical losartan for established corneal fibrosis
Open source

[N05] Starting evidence: Topical PDE4 inhibition for post-injury epithelial adhesion rescue
Open source

[N06] Starting evidence: Defined dual KLK5/KLK7 cyclic peptide for local Netherton treatment
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[N17] Starting evidence: DNase plus plasminogen to unlock retained tPA in resistant thrombi
Open source

[N18] Starting evidence: Selective A2B blockade during EVLP
Open source

[N22] Starting evidence: Post-onset cognate tRNA add-back in YARS1/GARS1 neuropathy
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[N23] Starting evidence: Exposure-matched piclidenoson in Lowe OCRL deficiency
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[N24] Starting evidence: Vanillic acid plus defined CoQ10 formulation in COQ9/selected COQ7 deficiency
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